

red blood cells. Also, the baby uses some of your iron reserves. If you have anemia, you may feel tired and breathless and look pale.

**Causes** Usually, anemia is due to iron deficiency. Occasionally, it's due to a lack of folic acid, vitamin B12, or rarer other problems. Bloodwork will help identify the cause.

**What might be done** Anemia is usually remedied with an iron supplement, but side effects may include constipation and black stools, so some women prefer to boost iron intake with diet (see [Iron-rich foods](#)).

**Weakened cervix** Rarely, a woman may have a weakened cervix, which can lead to a miscarriage after 13 weeks. Usually, these miscarriages are relatively painless: you may feel well and perhaps notice some extra vaginal discharge and then quite quickly miscarry the baby.

**Causes** Risk factors include a previous late miscarriage; cervical surgery; or a previous late termination of pregnancy.

**What might be done** If you're thought to be at risk, your doctor may suggest a scan to check the length of your cervix since a shortened cervix makes miscarriage more likely. If you had two or more late miscarriages, your doctor may recommend a cervical stitch put in at around 14–16 weeks to sew the cervix closed, which is left in until you're about 37 weeks pregnant.

**Obstetric cholestasis** is a rare condition affecting liver function that causes a buildup of bile acids in the bloodstream. The main symptom is severe itching without a rash, usually most intense on the palms of the hands and soles of the feet. It usually occurs after 28 weeks.

**Causes** Although not clear, genetic factors are likely involved since it tends to run in families. A woman who has the condition in one pregnancy will usually develop it in future pregnancies. Increased sensitivity to pregnancy hormones, which effect the way bile is processed, is also thought to play a role.

**What might be done** If you have itching of the palms of your hands and soles of your feet, having bloodwork done can check your liver function and bile acids. If labs are abnormal, you may be diagnosed with cholestasis of pregnancy. The itching is often treated with ursodeoxycholic acid. You will also need fetal monitoring; those with severe cholestasis may need to deliver preterm at 36 weeks due to risk of stillbirth.

**Gestational diabetes** develops due to pregnancy. In this condition, the pancreas produces insufficient insulin to move glucose (sugar) from the blood to be stored, resulting in high levels of glucose in the blood. It usually begins at 20–24 weeks. The risk is greater if you have a family history of late-onset diabetes or you've previously had a large baby, a stillbirth, or gestational diabetes.

**Causes** Insulin levels are inadequate due to the extra demands of the fetus and because hormones produced by the placenta block the effects of insulin.

**What might be done** Between 24 and 28 weeks, you'll be tested for gestational diabetes. If you have risk factors, you may take this test earlier. The test involves having a blood test in the morning after fasting, then drinking a special sugary drink and having a repeat blood sugar check one hour later. If you have high levels of glucose, you'll have to take a similar but longer test to confirm a diagnosis. If you do have gestational diabetes, you will be taught how to test blood glucose at home. In most cases, the diabetes can be controlled through diet and exercise. However, if these measures are ineffective you may need medication to reduce blood sugar levels or insulin injections. Extra scans may be done to check the baby's growth, and early induction (see [Induction of labor](#)) may be advised. If you've had gestational diabetes, ensure your weight is normal before becoming pregnant again.

**Polyhydramnios** is an excess of amniotic fluid. Symptoms include a stretched feeling in the abdomen; breathlessness, heartburn; swelling in the legs; and constipation. It is more likely with diabetes; twins; with an infection; or where there is a congenital